



DITO SA AMIN: A Quasi-Experimental Study Tracing Post-COVID-19 Health-Seeking Patterns in Geographically Isolated and Disadvantaged Areas

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Background of the Study

Local rural health facilities such as RHU and BHS were established to bring clinical healthcare closer to geographically isolated and disadvantaged areas (GIDAs). Evidence from the last two decades, however, point to the fact that healthcare platforms remain limited.



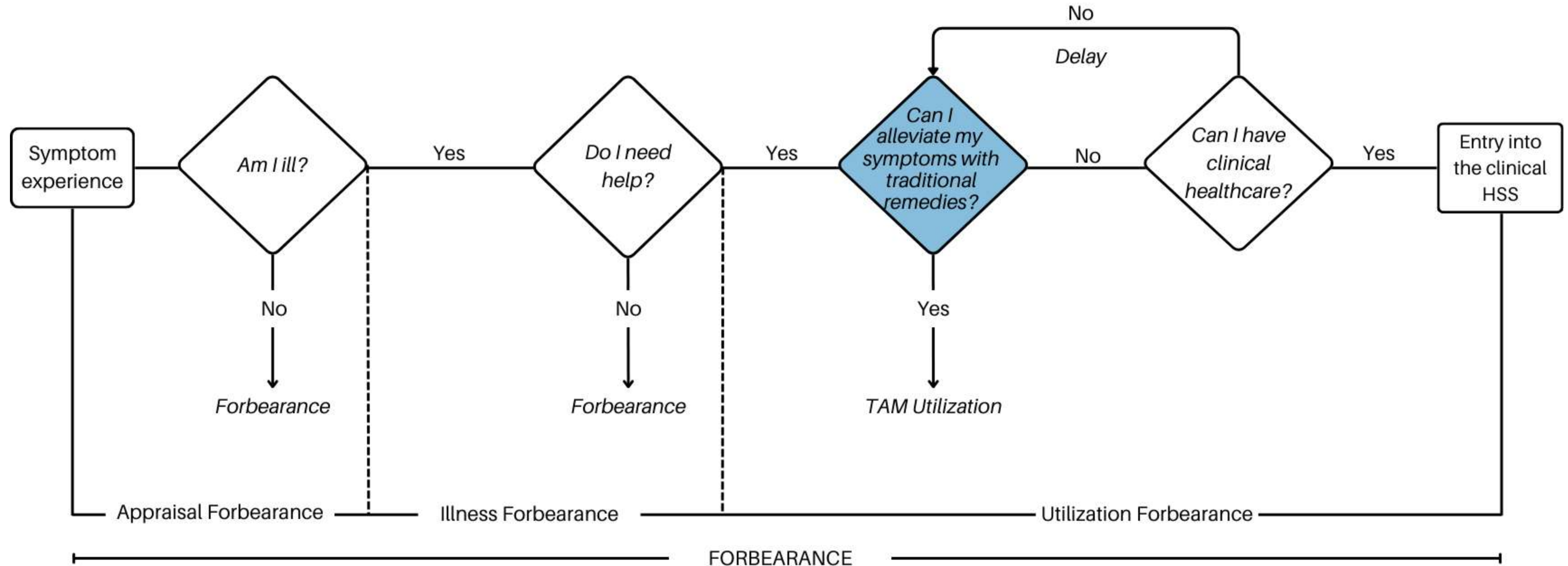
Objectives

This study investigated four non-equivalent communities in Nueva Ecija through statistical modeling to detect the health-seeking patterns of adult residents and determine which factor are associated to this pattern.

Conceptual Framework

Modifying the Stages of Delay by Safer et al. (1969), the Filipino Illness Forbearance Model was posited as a model of the pattern of health-seeking in rural GIDA communities. It emphasizes the concept of *pagtitiis* or forbearance, an indigenous coping mechanism.

Filipino Illness Forbearance Model



Sampling Method

odds ratio = 2.5

$\Pr(Y = 1 \mid X = 1) \mid H_0 = 0.50$

$\alpha = 0.05$

power $[1 - \beta \text{ err prob}] = 0.80$

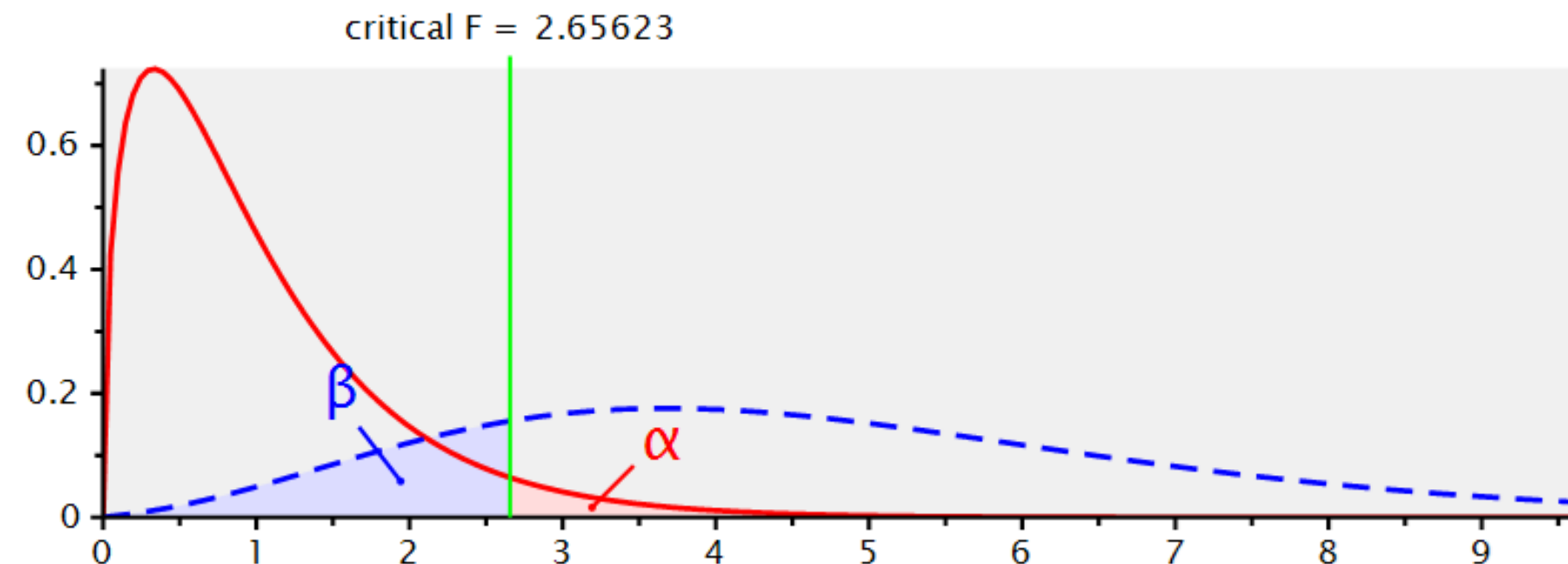
R^2 other $X = 0$

$X \text{ parm } \Pi = 0.50$

$n_0 = 164$

attrition rate allowance = 5%

n = 173



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Selection Criteria

- ✓ Physical Factor A
- ✓ Socioeconomic Factor A
- ✓ Socioeconomic Factor B

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n = 173

GIDA Communities	N ^{1,2}	n ³
Brgy. Salazar, Carranglan	300	29
Brgy. Langka, Palayan City	398	39
Brgy. Capintalan, Carranglan	699	69
Brgy. Putlan, Carranglan	365	36
Total	1,762	173

1 Number of households

2 Based on the 01 August 2015 Census (PSA, 2016)

3 Rounded to the nearest whole number





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1. What public healthcare platform is considered by a rural GIDA patient as first point of contact for primary and emergent care in post-pandemic context?
2. Is there a difference in clinical healthcare reliance among rural GIDA patients after the pandemic?
3. What factors predict a rural GIDA community's utilization rate of BHS, RHU, and TAM in post-pandemic context?
4. What factors affect a rural GIDA community's satisfaction level of BHS, RHU, and TAM in post-pandemic context?



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5. What strategies do rural GIDA patients employ to cope with illness symptoms?
6. Why do rural GIDA patients delay or avoid healthcare for their illness symptoms?
7. What do rural GIDA patients consider as the marker of necessity to seek healthcare in post-pandemic context?
8. Do rural GIDA patients consider themselves as an “other” in public healthcare provision?





**Paano magpagamot ang mga
taong walang pagamutan?**

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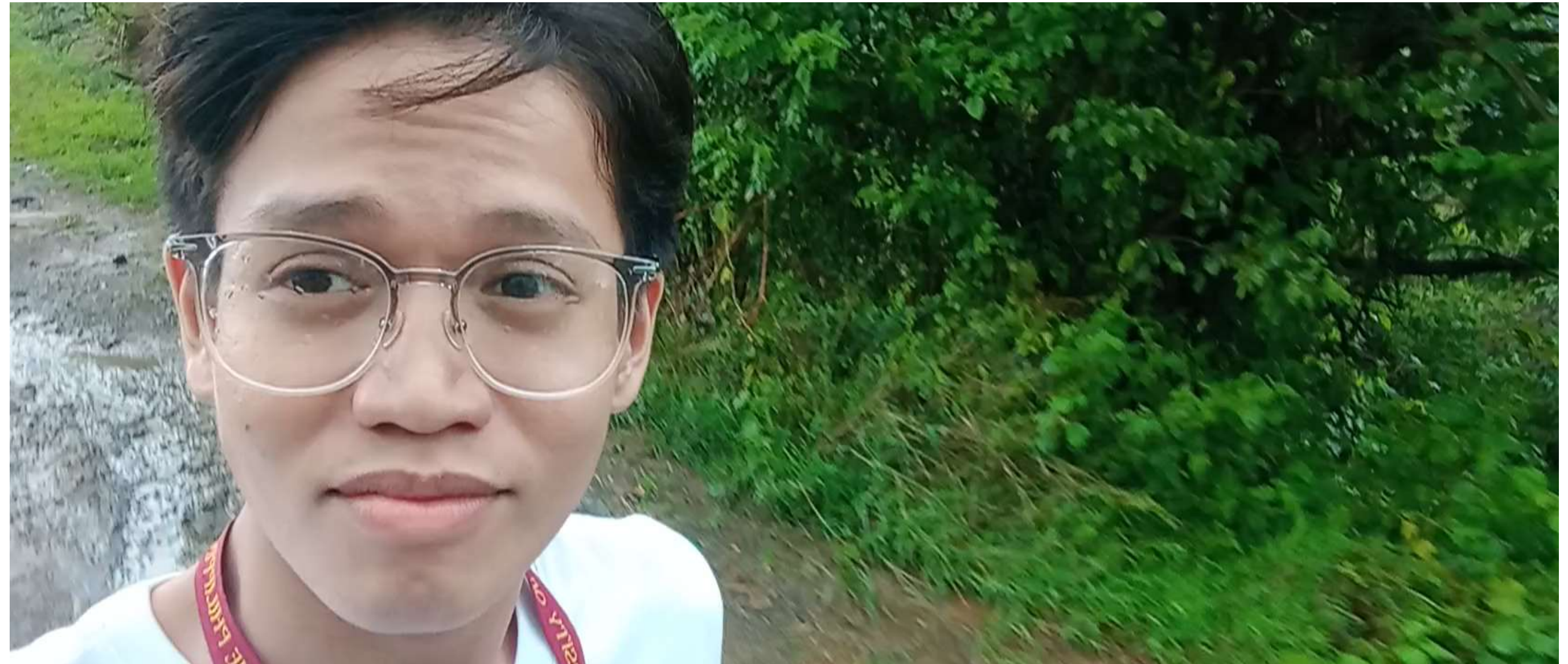
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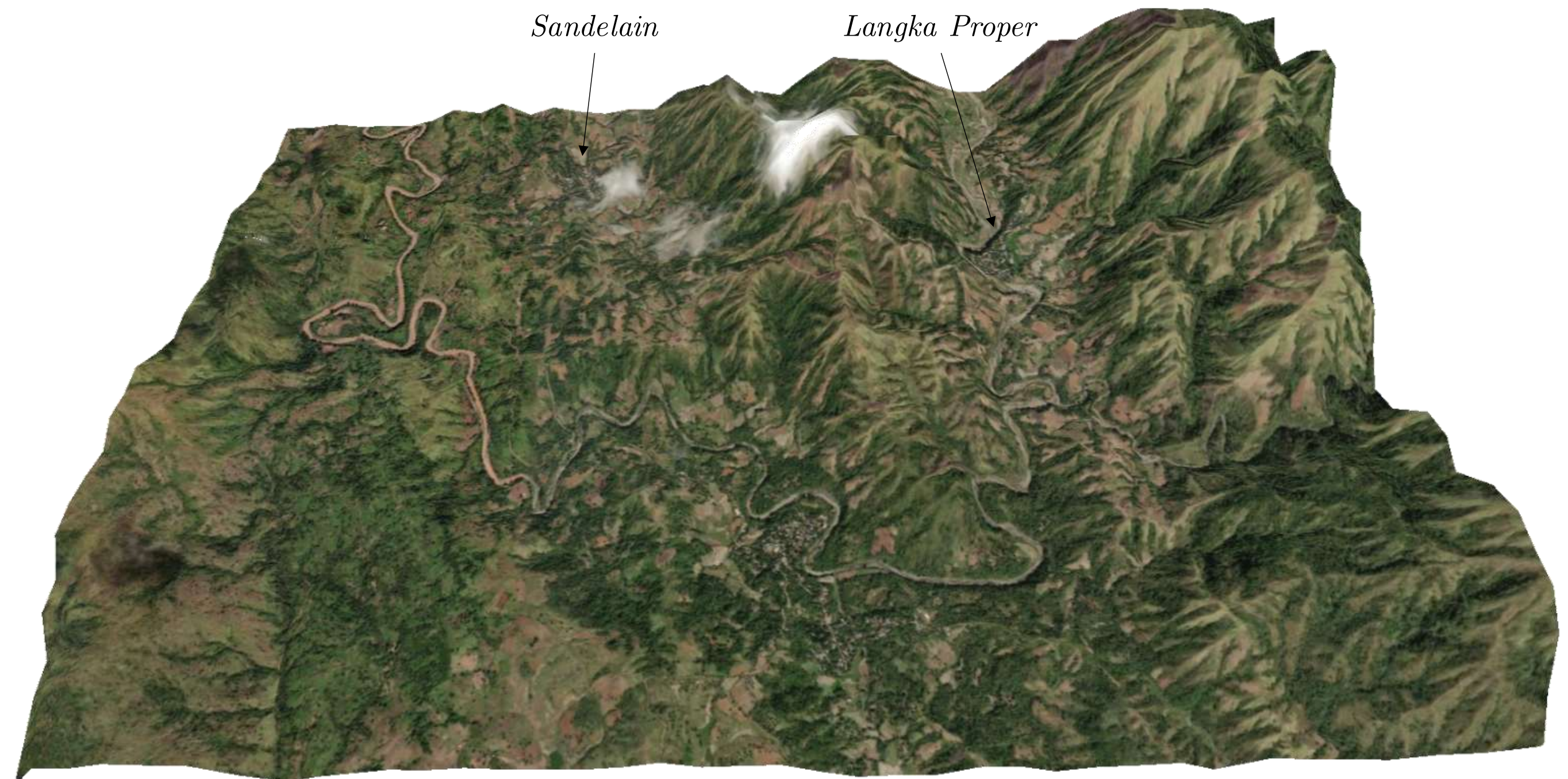
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Demographic Characteristics

GIDA Community	Age				Indigenous Status			Monthly Income			Proportion to n
	18-29	30-44	45-59	60+	IP	Non-IP	<10k	10k- 20k	20k- 30k	30k- above	
Brgy. Palale	3	12	9	5	16	13	27	1	1	0	16.96%
Male	0	0	4	3	1	6	6	0	1	0	4.09%
Female	3	12	5	2	15	7	21	1	0	0	12.87%
Brgy. Langka	11	15	7	6	19	20	39	0	0	0	22.80%
Male	3	5	2	1	6	5	11	0	0	0	6.43%
Female	8	10	5	5	13	15	28	0	0	0	16.37%
Brgy. Salazar	8	11	4	6	10	19	29	0	0	0	16.96%
Male	2	0	1	3	2	4	6	0	0	0	3.51%
Female	6	11	3	3	8	15	23	0	0	0	13.45%
Brgy. General Luna	16	37	13	10	43	33	74	2	0	0	43.28%
Male	3	5	2	5	8	7	15	0	0	0	8.77%
Female	13	32	11	5	35	26	59	2	0	0	34.51%
Total	38	75	33	27	88	85	169	3	1	0	100%
Male	8	10	9	12	17	22	38	0	1	0	22.8%
Female	30	65	24	15	71	63	131	3	0	0	77.2%



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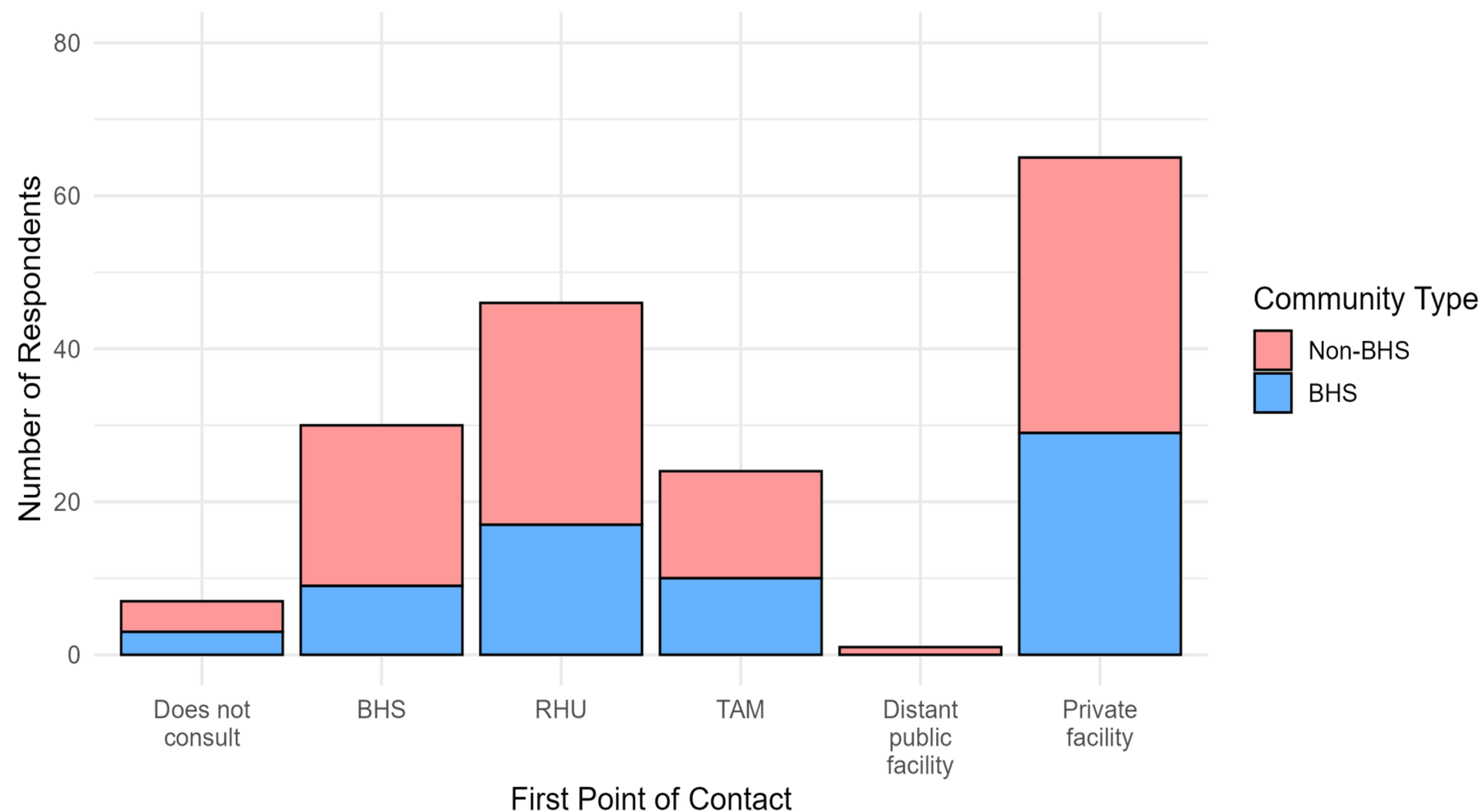
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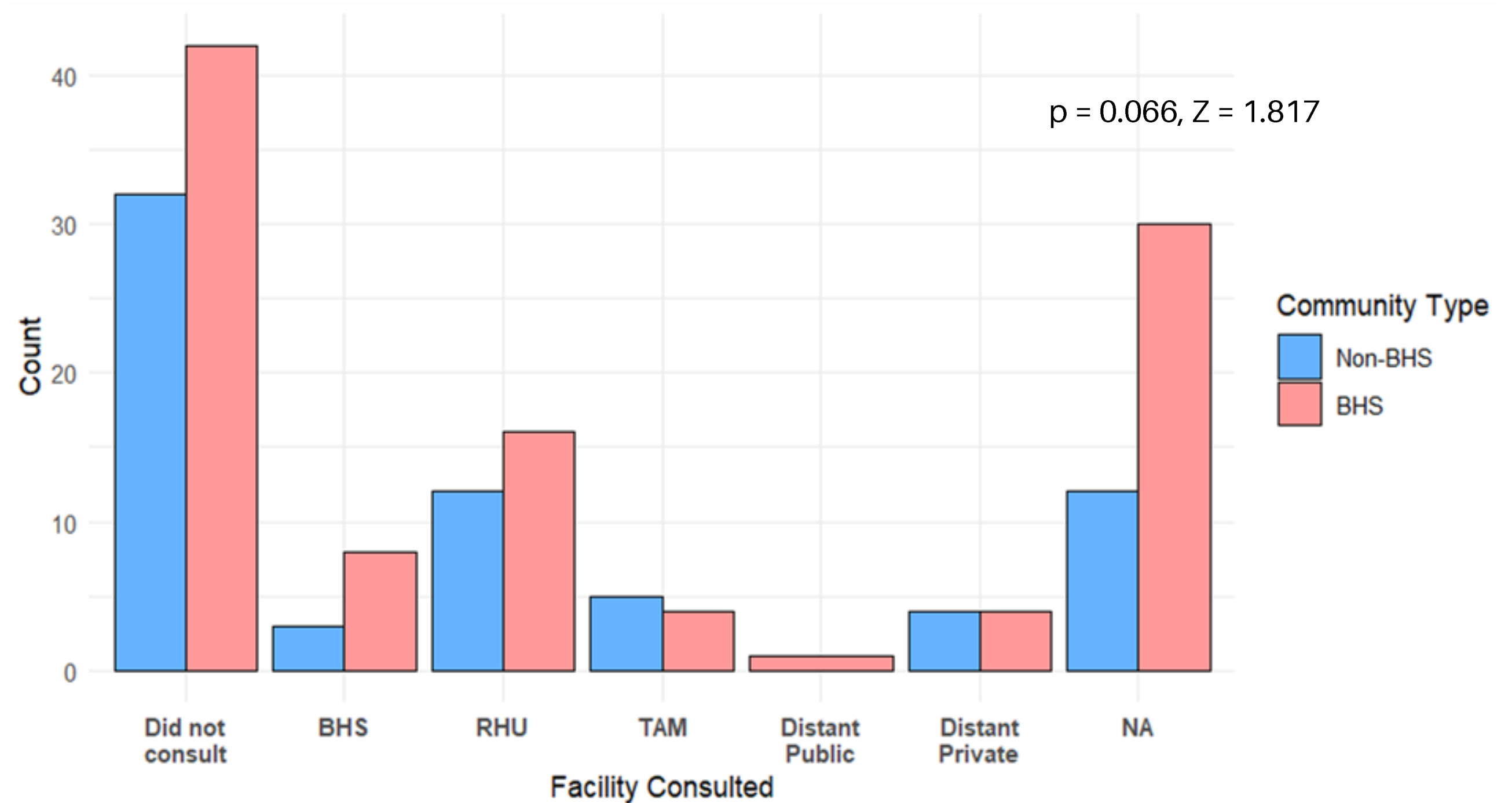
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First Point of Healthcare Contact



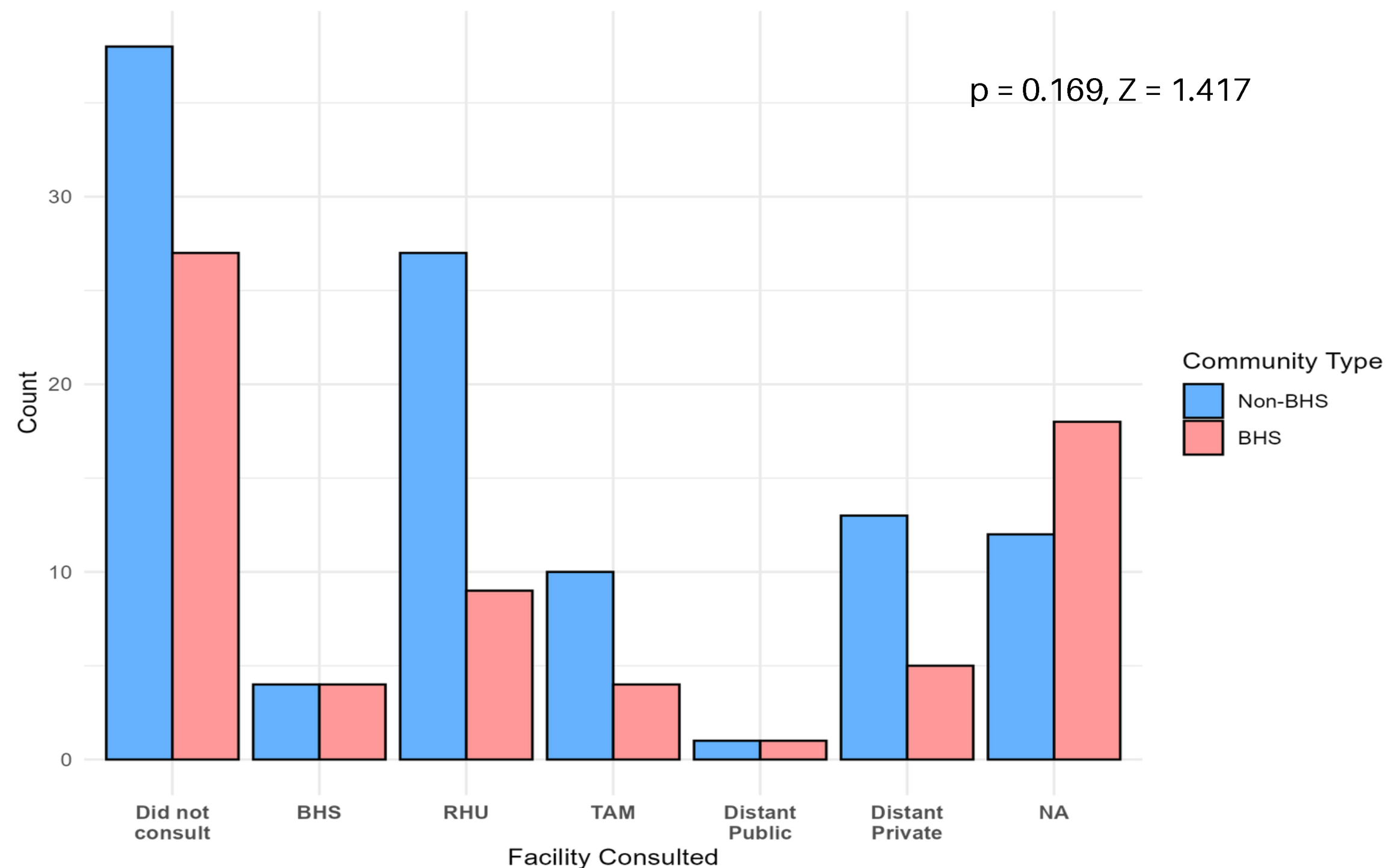
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Clinical Healthcare Reliance: Personal Illness



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Clinical Healthcare Reliance: Familial Illness



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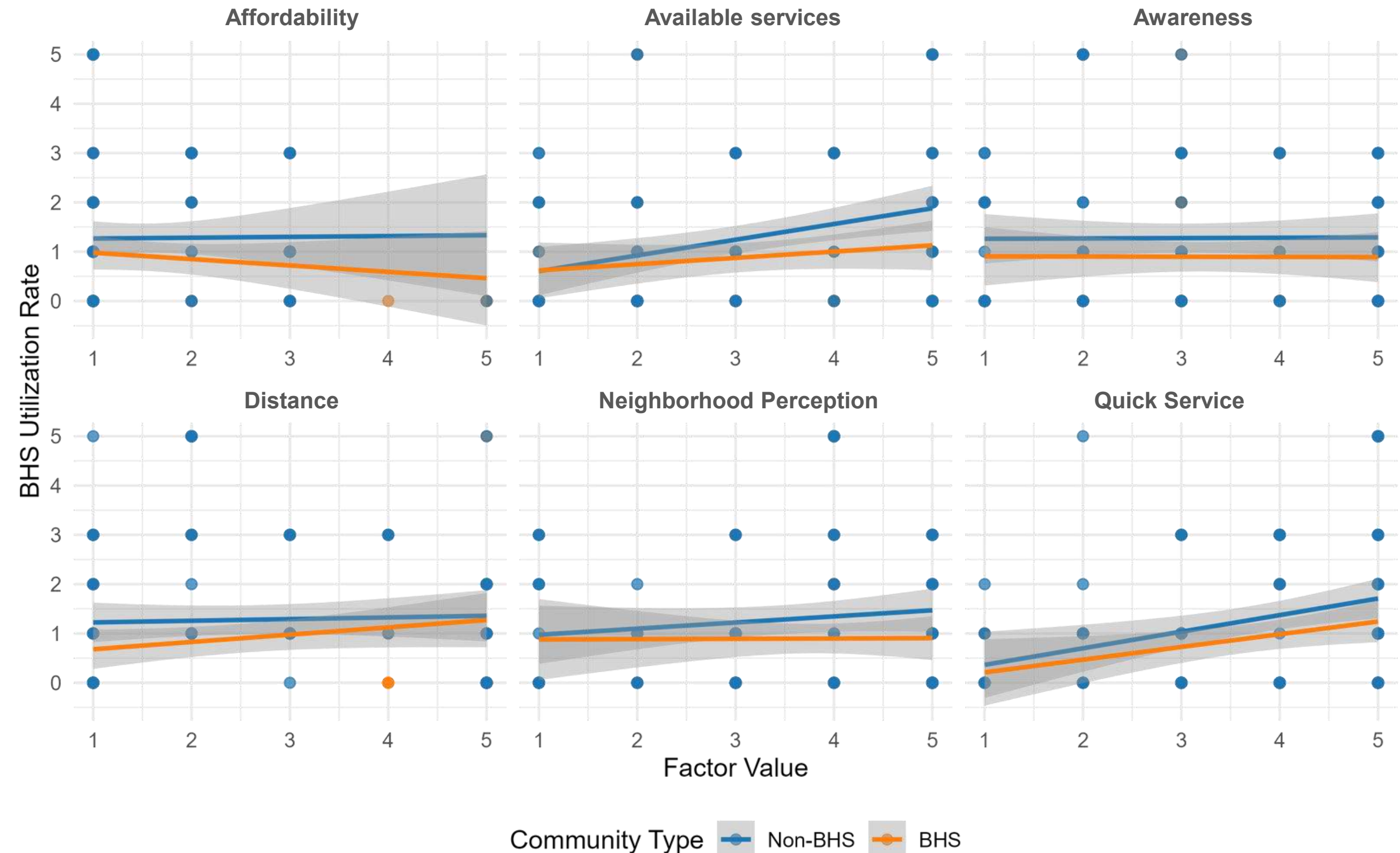
Nilagay siya sa may duyan na karga-karga ng mga kapitbahay namin pababa sa bundok. Sobrang tagal tapos umuulan din nun kaya delikado ang daan. Ayun, inabutan na siya sa daan. Pagdating namin sa baba, sinalubong kami ng ambulansya pero wala na siya.

Sussana (Participant 51)



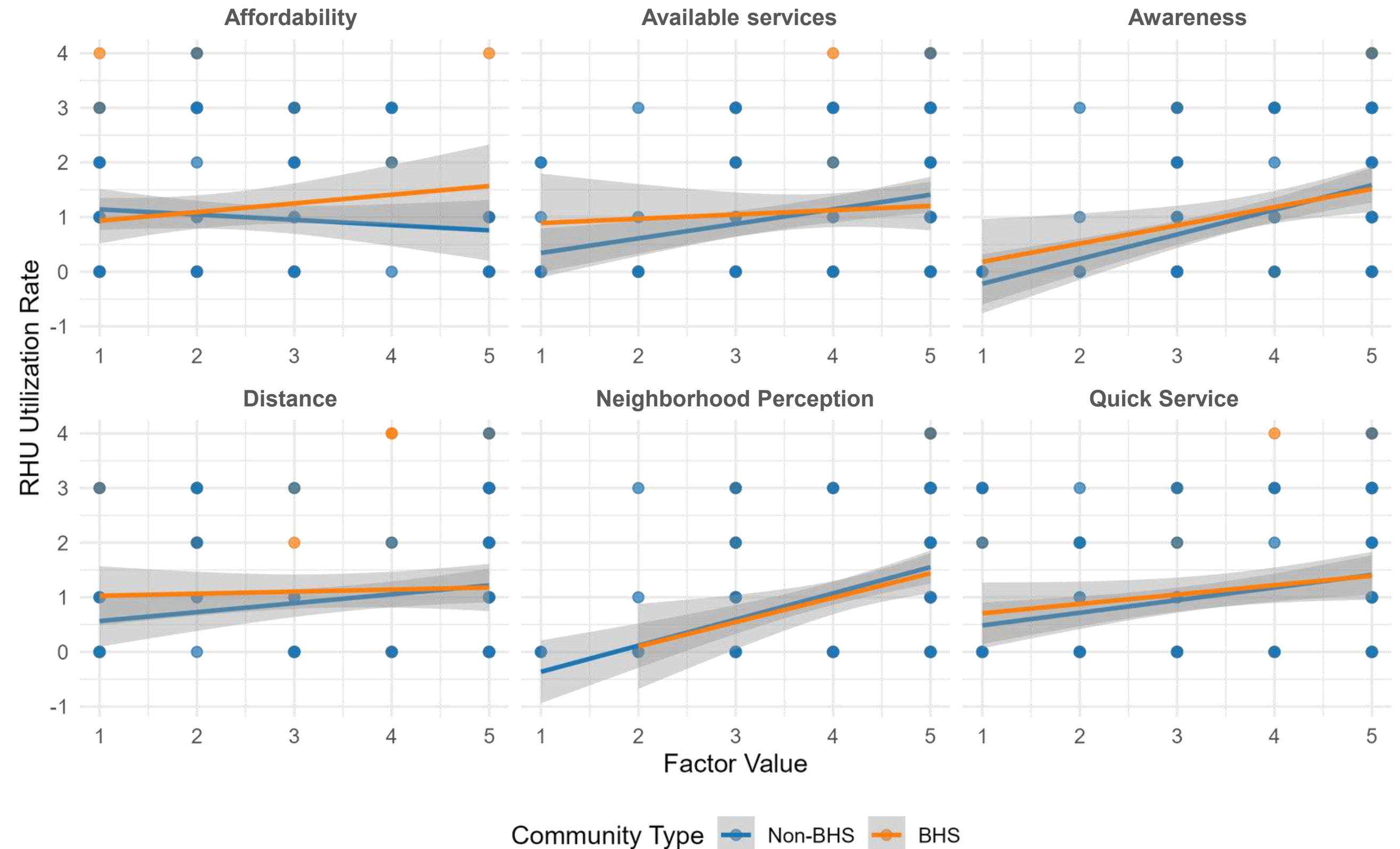
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Predictors of Utilization Rate: BHS



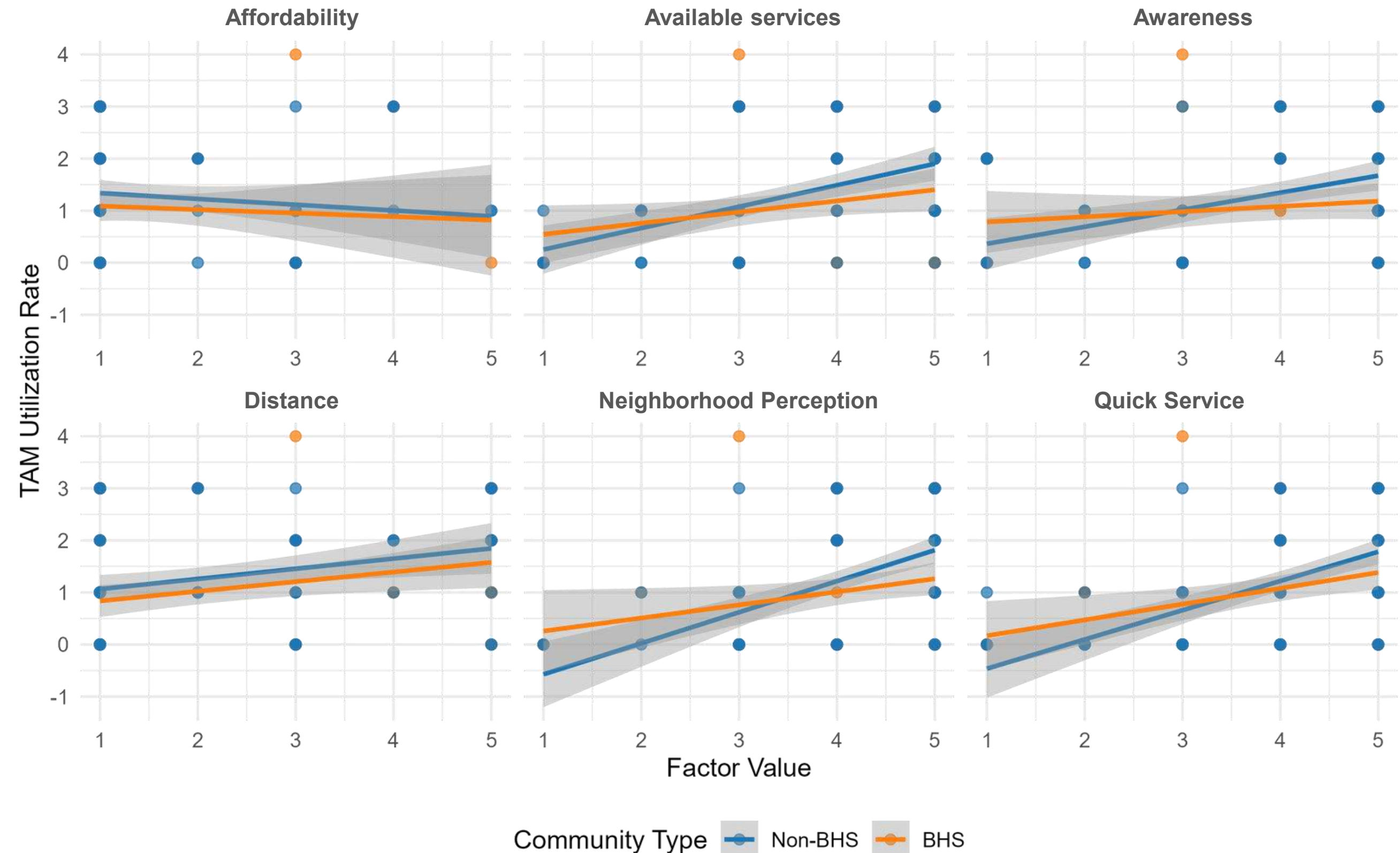
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Predictors of Utilization Rate: RHU



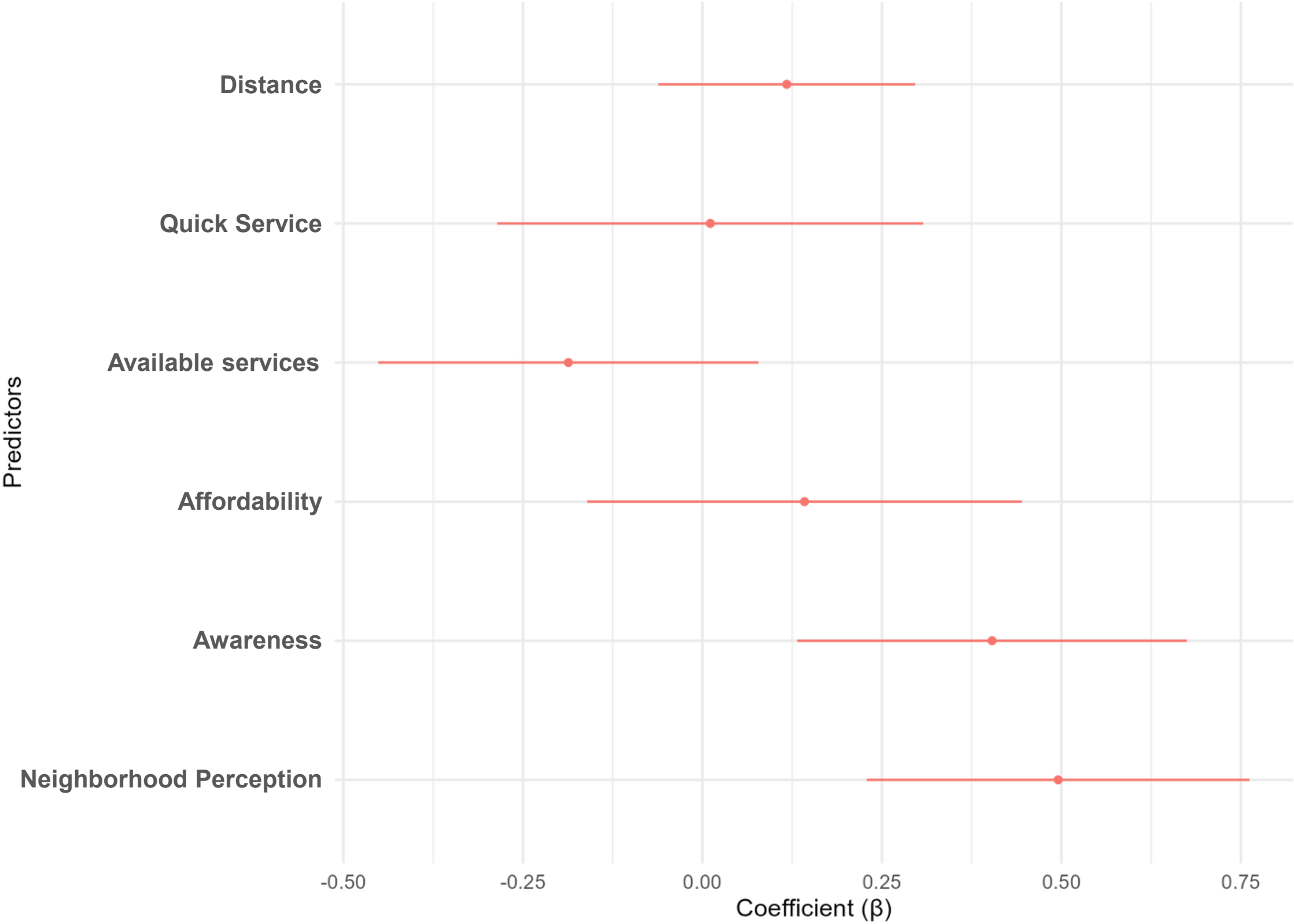
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Predictors of Utilization Rate: TAM



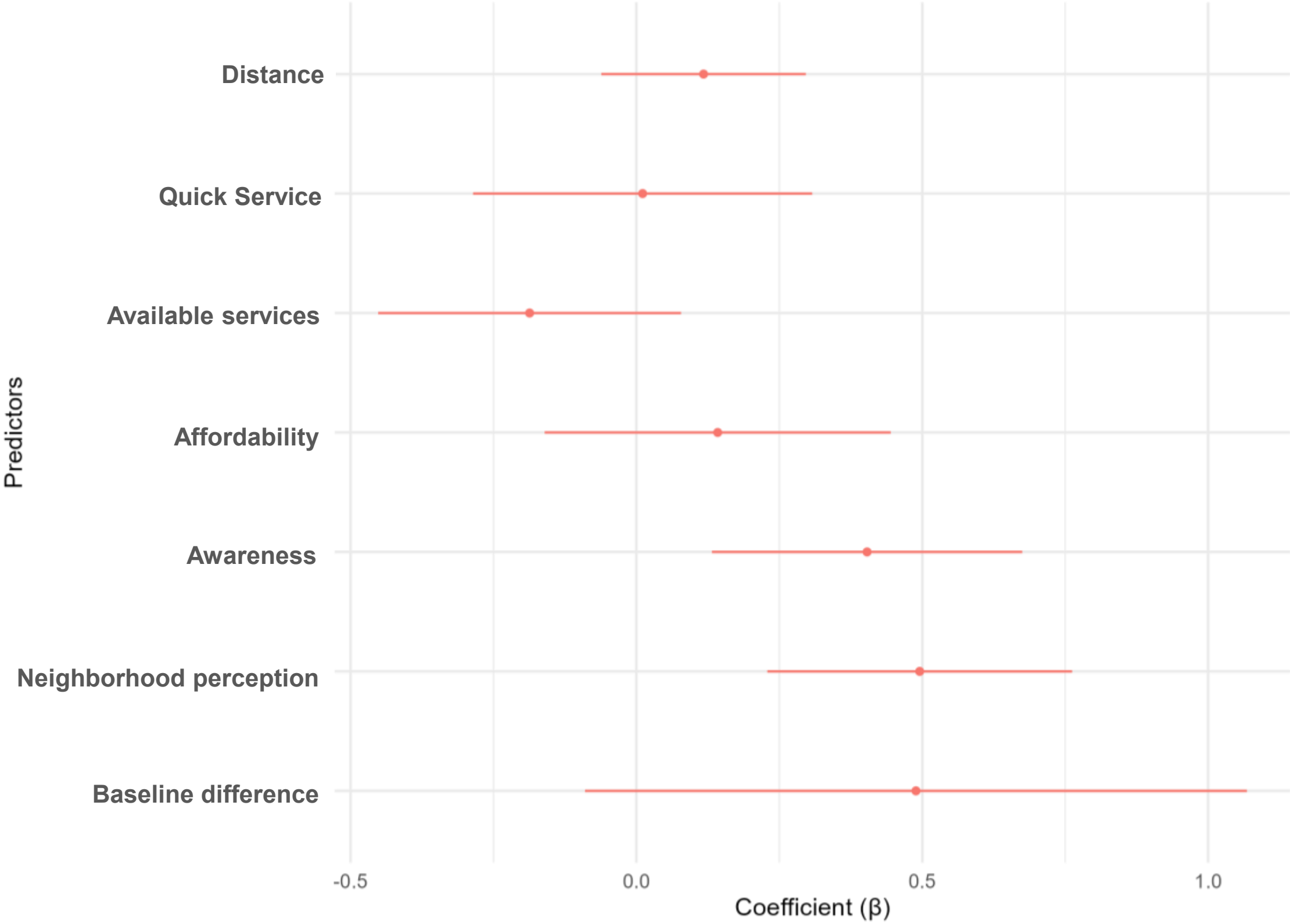
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Predictors of BHS Satisfaction Level in Non-BHS Communities



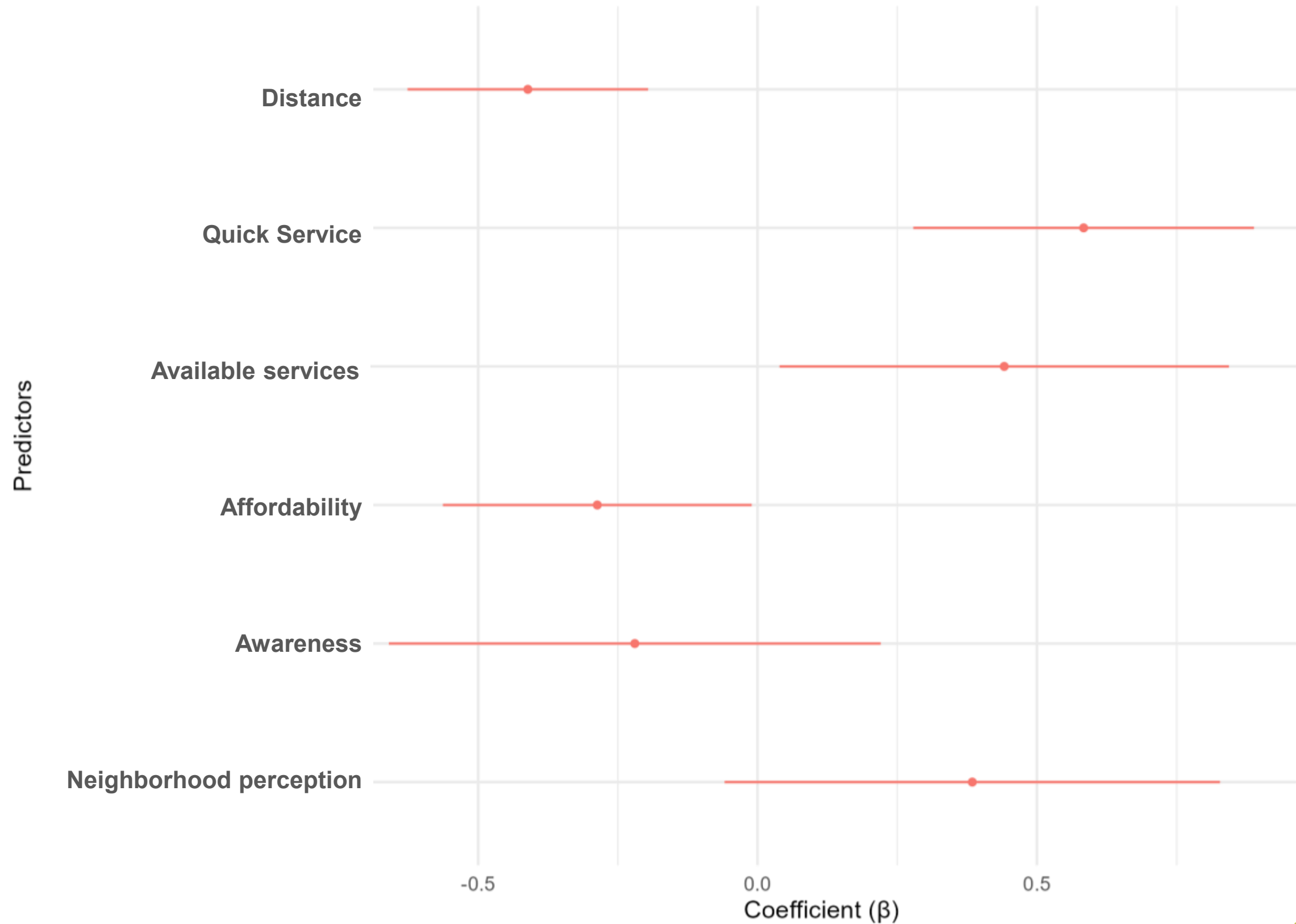
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Predictors of BHS Satisfaction Level in BHS Communities



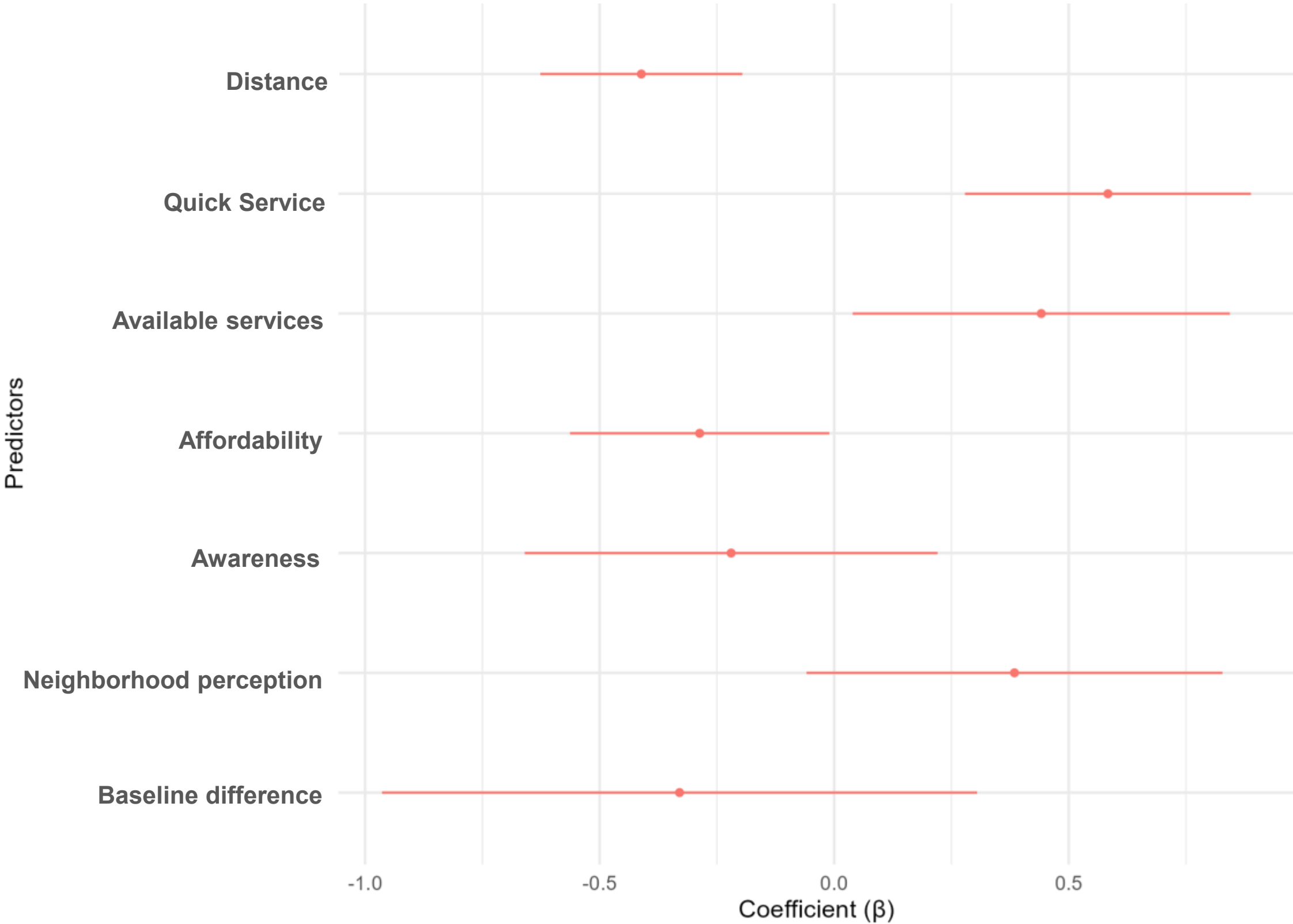
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Predictors of RHU Satisfaction Level in Non-BHS Communities



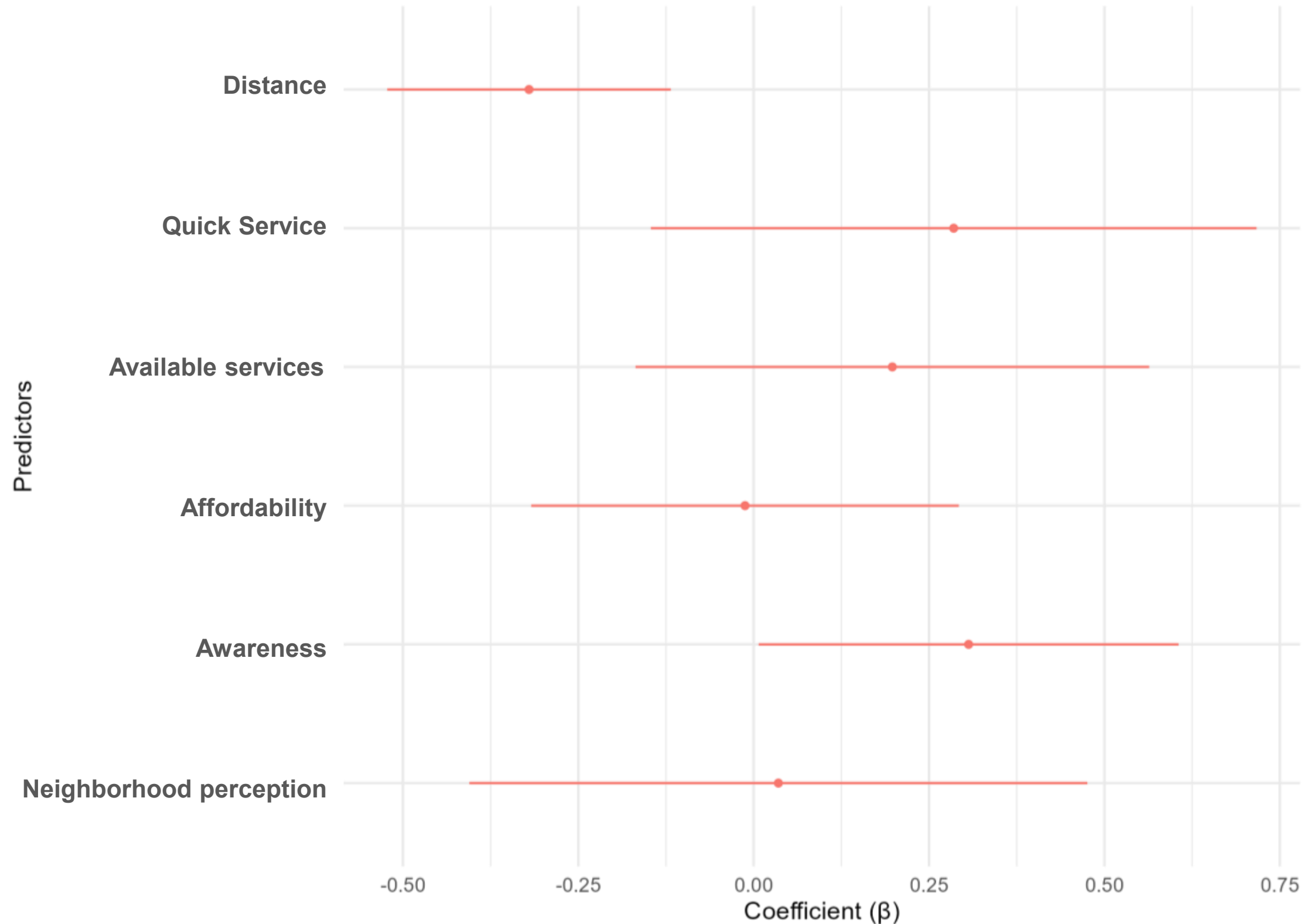
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Predictors of RHU Satisfaction Level in BHS Communities



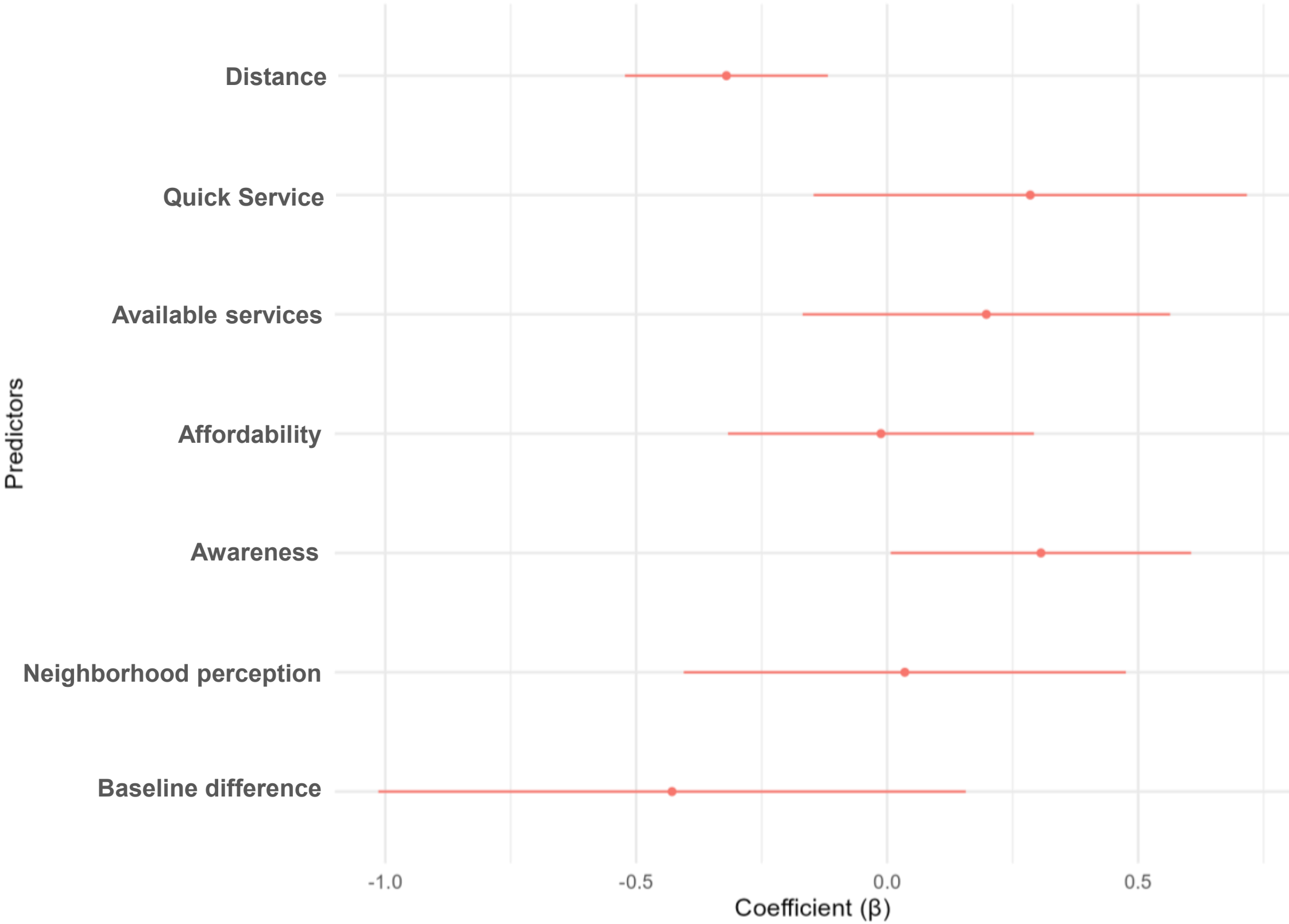
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Predictors of TAM Satisfaction Level in Non-BHS Communities



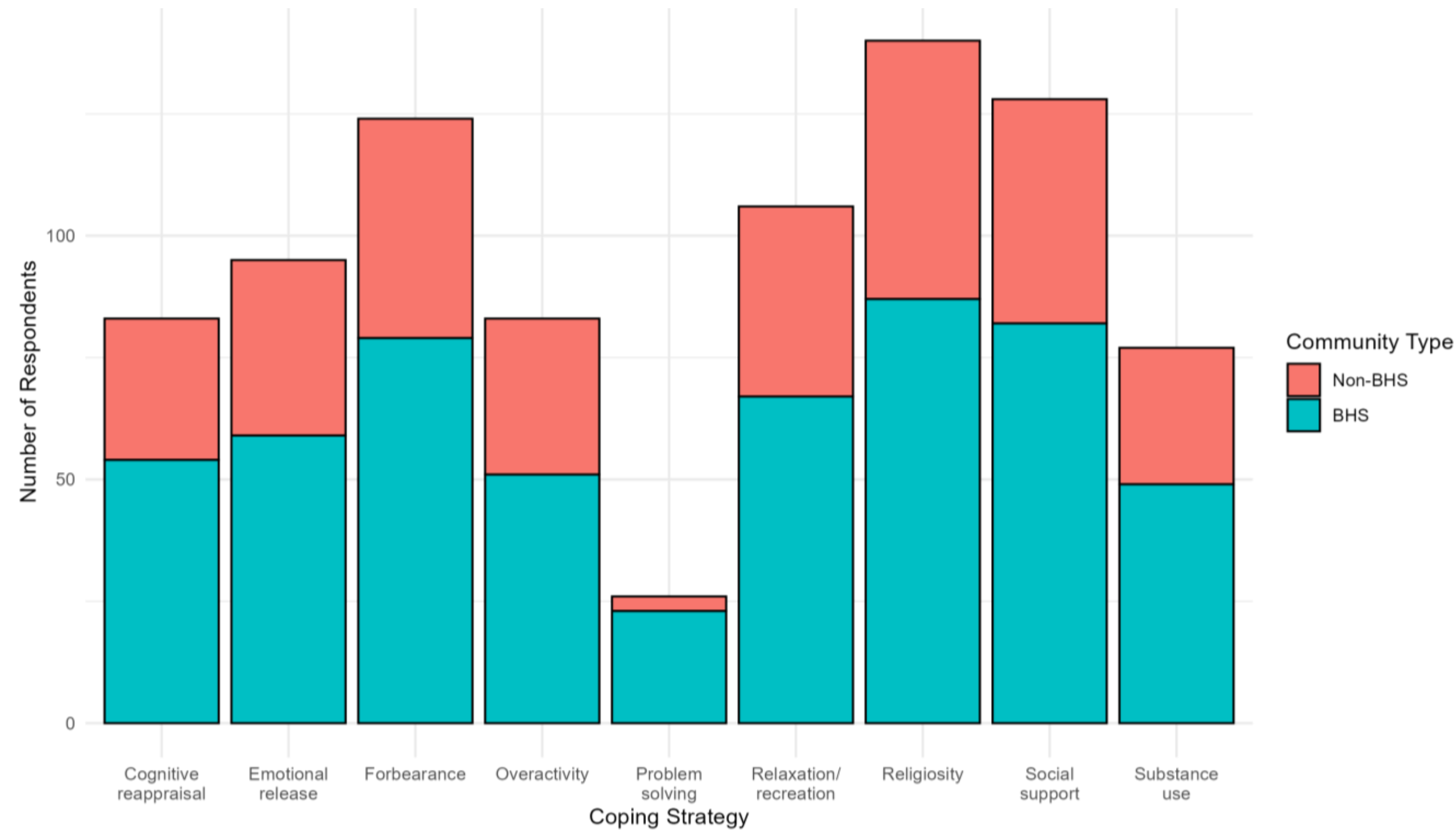
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Predictors of TAM Satisfaction Level in BHS Communities



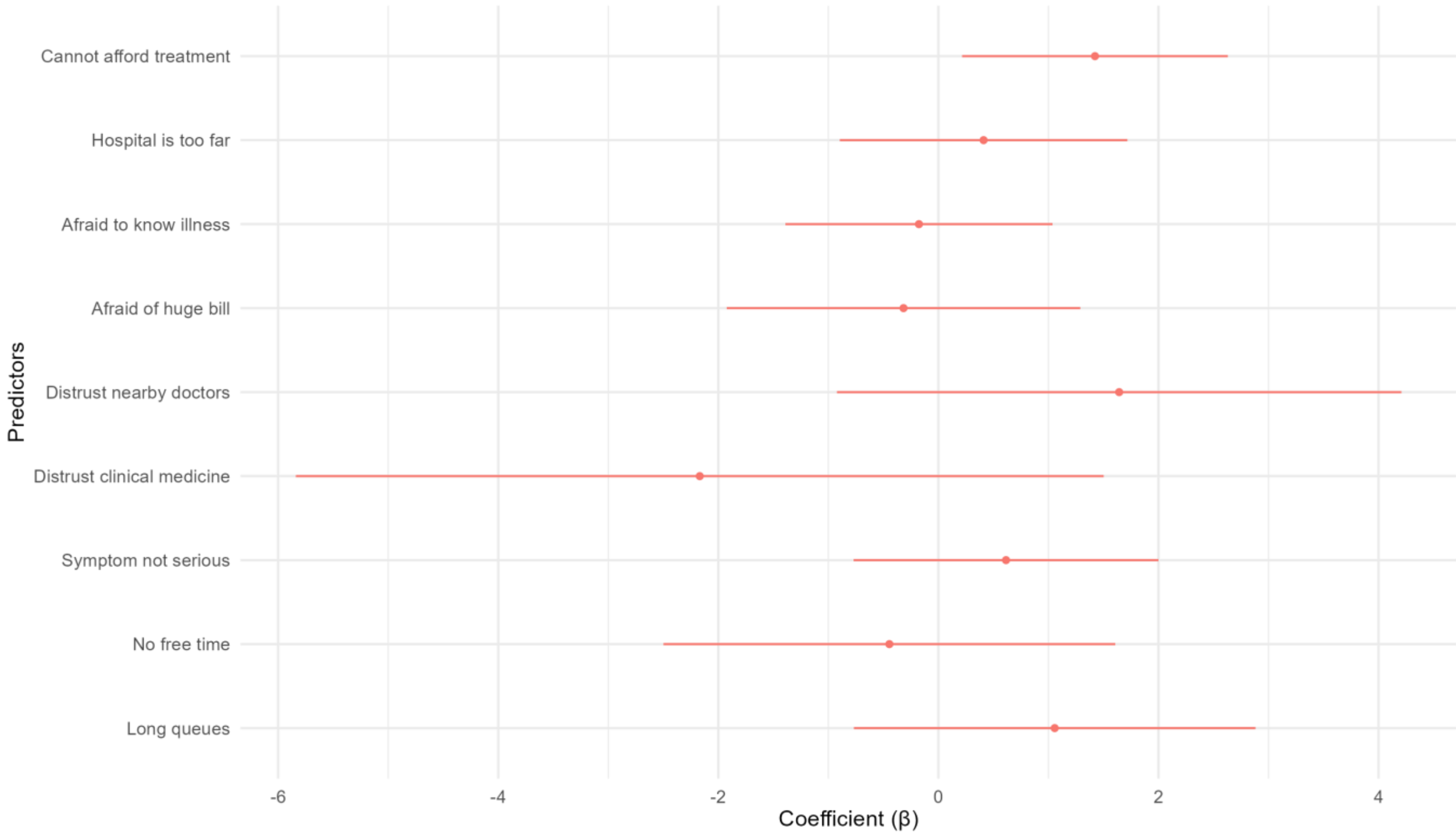
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Coping Strategies for Illness Symptoms



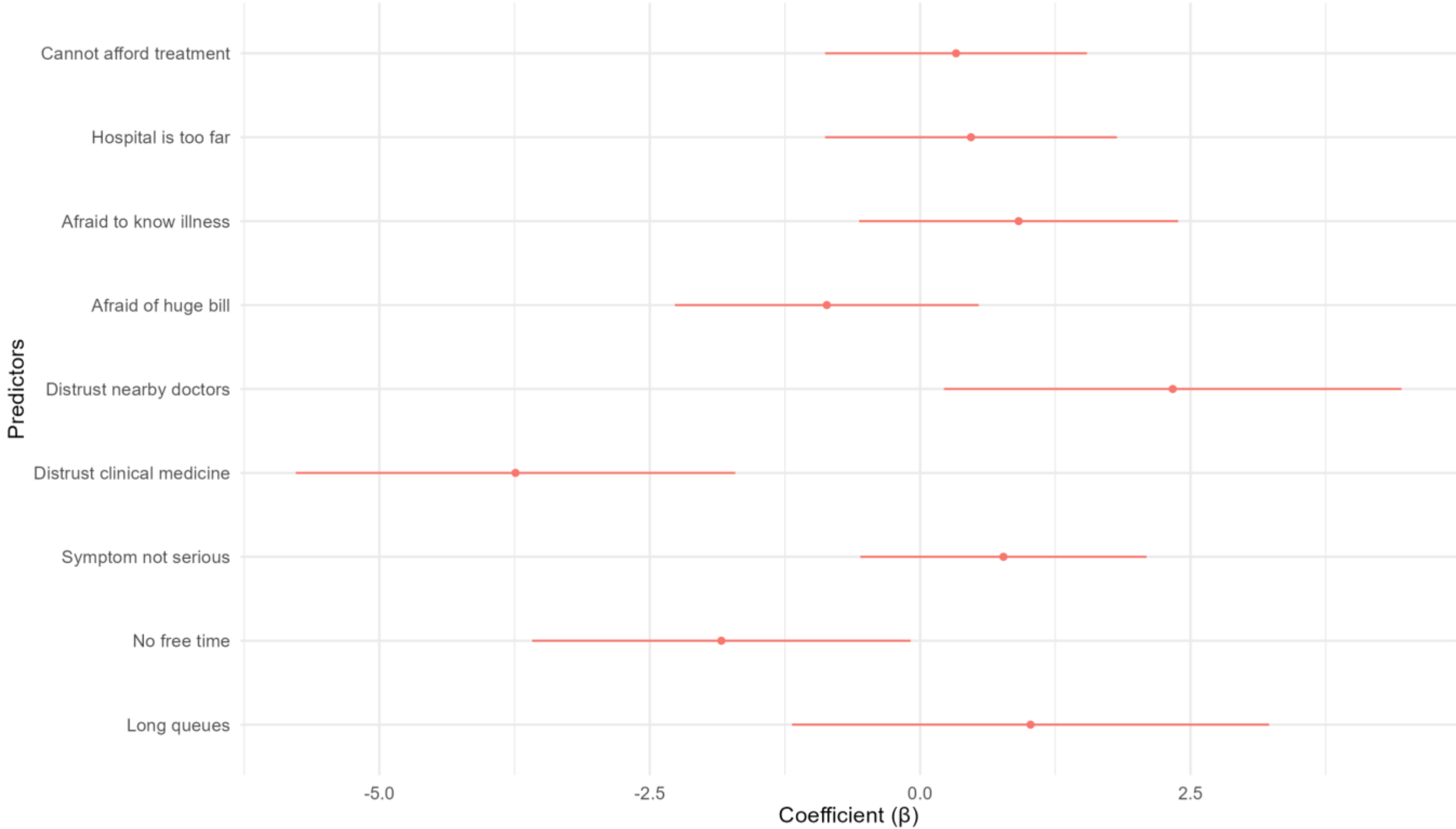
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Healthcare Delay and Avoidance: Non-BHS Communities



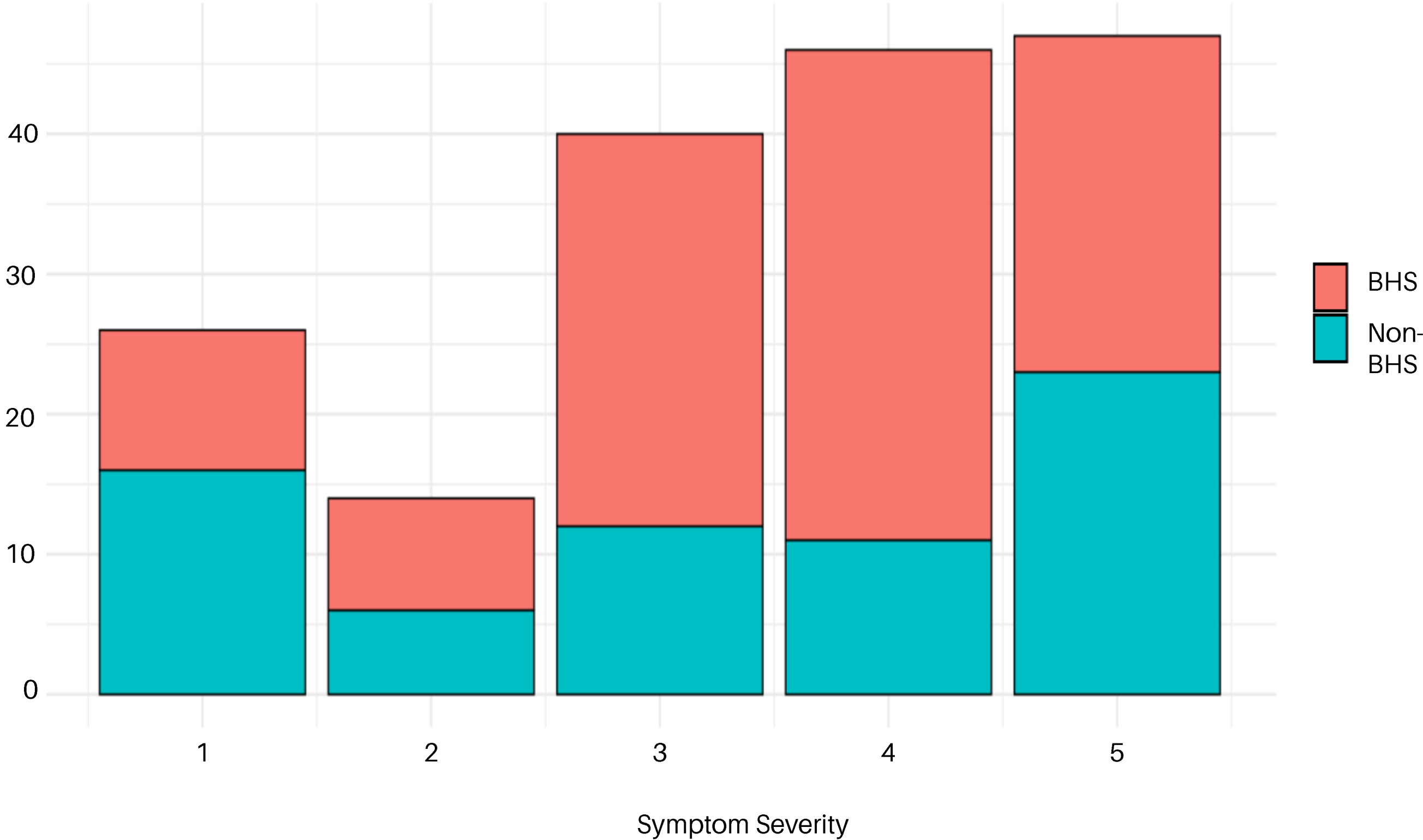
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Healthcare Delay and Avoidance: BHS Communities

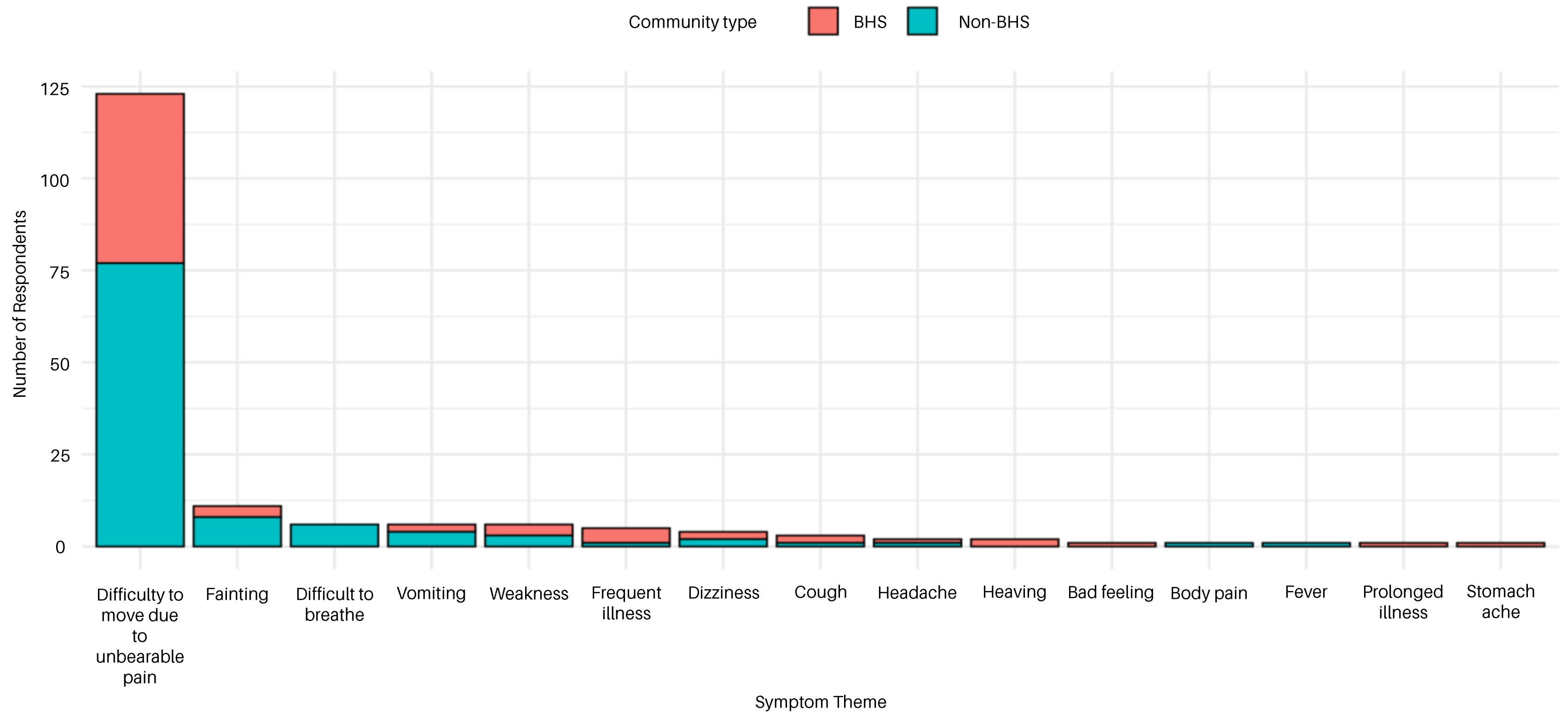


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Threshold for Health-Seeking



Threshold for Health-Seeking



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Loida (Participant 100)

X: Anong nararamdaman or iniisip mo kapag tinitiiis mo yung sakit mo?

Loida: Kinakabahan ako pero mas nauuna sa akin yung, 'Kaya ko 'to.'

X: Ano ibig sabihin nun? Ano ibig sabinin ng "Kaya ko 'to'?"

Loida: Yung halimbawa eh may sakit tapos syempre wala kang pampagamot, "Hmp, kaya ko 'to.'

X: Para ba siyang palakasan ng loob?

Loida: Oo, palakasan ng loob. Kapag sinabi mong kaya mo, edi magtitiis ka. Palakasan ng loob talaga ah. Minsan kasi walang-wala talaga tulad ngayon.



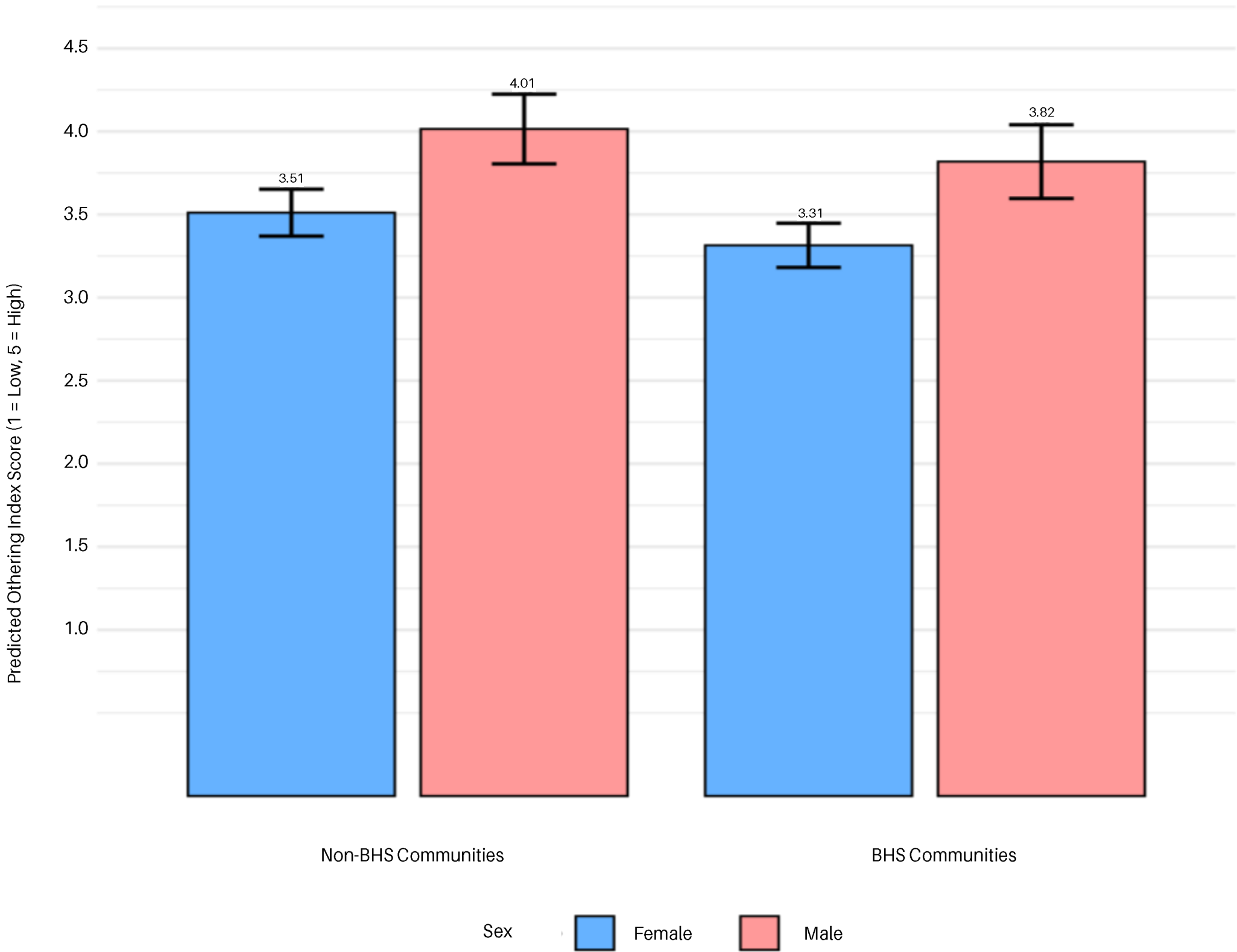
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An exploration of symptom perception shows that rural GIDA residents in four non-equivalent communities in Nueva Ecija delay their medical consults up until they cannot remedy it with simple forbearance strategies and traditional medicine.



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The personal, community, and economic status of the ill rural GIDA patient pushes them away from medical care, opting to forbear their symptoms and conditions.



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1. Equip local public healthcare facilities with human resources and tools by enlarging the allocated funds.
2. Establish healthcare facilities that are even closer to GIDA communities with enough personnel and equipment.
3. Hold frequent and regular information drive in barangays.



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4. Strengthen political will to back these recommendatory initiatives.
5. This study's findings and models may be applied in other GIDA communities in the Philippines to demonstrate its robustness and hopefully help the redesign of the HSS.
6. Bolster research in TAM and begin its more forward and robust integration into the clinical HSS





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Opening of the floor



Documentation





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